

• AN HSREP INITIATIVE IN DEVELOPMENT

The Prevention Adoption Gap — and a practitioner-built plan to close a measurable piece of it

Shared for professional review, feedback, and collaboration by colleagues across APHA and SOPHE.

THE PROBLEM

America's preventive services work; completion is what fails. People do not finish the screenings that already exist and are already recommended — a behavioral and operational failure, not a medical one. HSREP calls this the **Prevention Adoption Gap**.

8.5%

of adults 35+ get all recommended preventive services (HP2030 AHS-08)

63.5%

CRC screening vs. 72.8% national target (C-07)

\$47T

projected 15-year U.S. chronic-disease cost (PFCD)

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post-pandemic screening regression, largest among minority groups (JAMA HF, 2024)

THE PLATFORM BEHIND IT

HSREP (Health System Resilience & Economic Protection) is an advocacy publishing platform built from a two-country MPH capstone campaign: nine published pieces across The Daily Star (Bangladesh) and The Eastern Echo (Michigan), reaching **57,274 documented impressions** at zero ad spend, with a locked, reproducible analytics methodology. The platform consolidates the arguments, evidence, media packages, and engagement data — and now carries the applied initiative below.

THE INITIATIVE, IN BRIEF

The **Prevention Adoption Initiative** packages behavioral design into a replicable toolkit for safety-net settings, built on three pillars:

- ✓ Service redesign & friction reduction (defaults, mailed at-home testing)
- ✓ Data infrastructure as intervention (dashboards that surface who drops out, and where)
- ✓ Implementation enablement (training so sites sustain it themselves)

First demonstration: a 12-month colorectal cancer screening completion pilot at one Southeast Michigan safety-net site — one disease, one population, one locked primary outcome, evaluated with RE-AIM against the site's own baseline.

Every intervention lever is CDC Community Guide-recommended, and the outcome is a measure sites already report to HRSA and are paid on under Medicaid quality programs.

WHAT WE'RE ASKING OF COLLEAGUES

Three things, each small: **(1) critique** — review the pilot design and tell us where it's weak; **(2) connections** — introductions to safety-net leaders, FQHC quality teams, or Primary Care Association contacts in Michigan; **(3) a platform** — invitations to present or discuss the Prevention Adoption Gap at section meetings, roundtables, or conference sessions.



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